GAMSA · Gender-Affirming Medicines South Australia

# Blood test guide: Testosterone gel

Testosterone gel (Testogel 1%, Testavan 2%) · gamsa.au/monitoring

**Key points** Testogel: in the morning before that day's gel, about 10 to 20 nmol/L. Testavan: 2 to 4 hours after the gel, 17.3 to 36.4 nmol/L.

| Applies to me | Test | When | Aim | Talk to your prescriber if |
|---|---|---|---|---|
| ☐ | **Testosterone level (1% gel: Testogel)** | In the morning, before that day's gel, after at least a week on the same dose | About 10 to 20 nmol/L before the day's gel Other guidance (2019 position statement): 10 to 15 nmol/L | It is below about 10 nmol/L or above about 20 nmol/L |
| ☐ | **Testosterone level (2% gel: Testavan)** | 2 to 4 hours after putting the gel on. About 2 and 5 weeks after starting or a dose change, then at each review | 17.3 to 36.4 nmol/L at that time (for a full dose). The 10 to 20 nmol/L trough figure does not apply | It is below 17.3 nmol/L or above 36.4 nmol/L: your prescriber may change the dose by one pump |
| ☐ | **Full blood count (haemoglobin and haematocrit)** | Before starting, then at each review. Checked more closely if you take an SGLT-2 inhibitor (such as empagliflozin or dapagliflozin) | Haematocrit below 0.50 L/L (50%), read against the male range | It is 0.50 L/L (50%) or more: smoking, sleep apnoea and lung or kidney problems are checked, and the interval or dose may be changed, or a gel or cream used instead |
| ☐ | **Liver function, lipids and glucose** | Before starting, then at each review or as your prescriber decides | Within the lab's range | Your prescriber flags a change |
| ☐ | **Pregnancy test** | Before starting, if a pregnancy is possible. Testosterone is not contraception | — | You could be pregnant: testosterone can harm a developing baby, so tell your prescriber straight away |

A guide to what to expect, not a schedule to follow on your own. Your prescriber sets the timing.

## Reading your results

- Lab reports mark results as high or low using the sex on your Medicare or lab record. On hormones, that range may not fit your body. In the first year your prescriber may ask the lab for both the male and the female ranges.
- A single flagged result is a reason to talk with your prescriber, not to change a dose on your own.
- Units: Australian labs report oestradiol in pmol/L, testosterone in nmol/L, and haematocrit as a fraction (0.50 L/L is the same as 50%).

### Things that can make a result look wrong

- A sample taken at a different time after the dose from the one your prescriber asked for.
- Oestradiol gel on the skin where the blood is taken: ask for the sample from the arm you did not put gel on. Doing the same with testosterone gel or cream is a sensible precaution, though the guidelines do not say it.
- Haemoglobin and haematocrit: read against the range for your affirmed gender once hormones have had time to work (about 3 months on testosterone, about 6 months on oestradiol). On testosterone, the male range is used.
- Kidney function (eGFR) is worked out from your creatinine, age and recorded sex, so the change over time tells more than one result.
- Iron studies: the range for your affirmed gender, unless you are pregnant or have periods (then the female range is used).

Sources: AusPATH 2026 pp21, 22, 35 (Table 7), 36, 42, 55; Testogel PI s4.2; Equinox guide 2020 pp6, 7; Testavan PI s4.2; Endocrine Society 2017 Table 14; 2019 position statement Contraception, Monitoring; Reandron 1000 PI s4.4. Full references and detail for clinicians: gamsa.au/monitoring.

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